PEER REVIEW HISTORY

BMJ Open publishes all reviews undertaken for accepted manuscripts. Reviewers are asked to complete a checklist review form (**http://bmjopen.bmj.com/site/about/resources/checklist.pdf)** and are provided with free text boxes to elaborate on their assessment. These free text comments are reproduced below.

ARTICLE DETAILS

TITLE (PROVISIONAL)	Connecting healthcare with income maximisation services, and their financial, health and wellbeing impacts for families with young children: A Systematic Review Protocol.
AUTHORS	Burley, Jade; Price, Anna; Parker, Anneka; Zhu, Anna; Eapen, Valsamma; Contreras-Suarez, Diana; Schreurs, Natalie; Lawson, K; Lingam, Raghu; Grace, Rebekah; Raman, Shanti; Kemp, Lynn; Chota, Sumayya; Goldfeld, Sharon; Woolfenden, Susan; Samir, Nora

VERSION 1 – REVIEW

REVIEWER	Dani Filc Ben-Gurion University of the Negev, Politic and Government
REVIEW RETURNED	16-Sep-2021

GENERAL COMMENTS	The proposed research is interesting and the proposal is sound. In my opinion there are a few points that should be taken into consideration. 1)There is a contradiction between the statement that no research has "focused exclusively on referral parhways from healthcare to income-maximisation services" and the proposal to do meta-analysis of existing research. 2) Criteria of inclusion seem to me to loose, since "there will be no limit on sample size for study desings". I think it would be better to do metaanalysis of studies with samples that are statistically sound. 3) The paper aims to deal with social determinants of health, but the second outcome "financial literacy of families", is not a social determinant of health.

REVIEWER	Anup Karan Indian Institute of Public Health, Delhi (IIPHD), Public Health Foundation of India, HEalth Economics
REVIEW RETURNED	27-Sep-2021

GENERAL COMMENTS	This study protocol aims to conduct a systematic review of literature on assessing the impacts of health care-income maximisation models on the financial situation of families with children in the age group of 0-5 years in the context of high-income countries. This is a very important and timely planned study which will help synthesize the roles of financial counselling and advises in addressing the issues related to health related financial burden on families in general and child healthcare in particular. I find the systematic review study well planned with robust methodology as study aims to use most standard guidelines. However, I have a few minor comments which will help the article better reading and clearer understanding of the paper. My main comments are as follows:

	1. Authors may like to a little expand on any theoretical model of impact of healthcare-income maximisation models of care on family finances to better convey the message and the context to the readers. Exact pathways may evolve later after the review but likely channels could be indicated briefly. Although authors have mentioned a few indicative reasoning and have also referred to a few existing literatures (line # 26-37), I guess some more details on explaining this framework will help. 2. In the section on 'Study Eligibility' authors mention that "studies will be ineligible if there is a focus on cash transfer programme" (line # 44-45). Why so? Does cash transfer not qualify for income maximisation tool of families? Need some clarifications on this? 3. Why do authors consider child health and wellbeing as a secondary outcome and not primary? I guess main outcomes of the planned review should be both financial as well as health. Need some rationale of dividing the outcomes into the two categories.

VERSION 1 – AUTHOR RESPONSE

Reviewer 1: The proposed research is interesting and the proposal is sound. In my opinion there are a few points that should be taken into consideration.

Response: Thank you for your feedback. We have addressed your comments in the table below. Added text is highlighted in red.

Feedback	Response
There is a contradiction between the statement that no research has "focused exclusively on referral pathways from healthcare to income-maximisation services" and the proposal to do meta-analysis of existing research.	Thank you for your comment. We have removed the statement that no research has focused exclusively on referral pathways from healthcare to income-maximisation services.
Criteria of inclusion seem to me to loose, since "there will be no limit on sample size for study designs". I think it would be better to do metaanalysis of studies with samples that are statistically sound.	Thank you we have removed from the text that "there will be no limit on sample size for study designs". In our exclusion criteria we have highlighted that qualitative research and case series are excluded.
The paper aims to deal with social determinants of health, but the second outcome "financial literacy of families", is not a social determinant of health.	Thank you for your comment. Upon consultation with financial counselling experts, they highlighted the importance of financial literacy in income management and how families understand and use their income, therefore we believe it is also an important outcome to consider in relation to the social determinants of health.

Reviewer 2: This study protocol aims to conduct a systematic review of literature on assessing the impacts of health care-income maximisation models on the financial situation of families with children in the age group of 0-5 years in the context of high-income countries. This is a very important and timely planned study which will help synthesize the roles of financial counselling and advises in addressing the issues related to health related financial burden on families in general and child healthcare in particular. I find the systematic review study well planned with robust methodology as study aims to use most standard guidelines. However, I have a few minor comments which will help the article better reading and clearer understanding of the paper.

Response: Thank you for your feedback. We have addressed your comments in the table below. Added text is highlighted in red.

Feedback	Response
Authors may like to a little expand on any theoretical model of impact of healthcare-income maximisation models of care on family finances to better convey the message and the context to the readers. Exact pathways may evolve later after the review but likely channels could be indicated briefly. Although authors have mentioned a few indicative reasoning and have also referred to a few existing literatures (line # 26-37), I guess some more details on explaining this framework will help.	Thank you for your comment. We have now included more details and included additional literature in the manuscript. The exact causal mechanisms underlying these effects are complex, however two theoretical model attempt to provide insight into the mechanisms. According to the investment model, families invest additional economic resources into their children, addressing material deprivation and improving health and developmental opportunities (12, 13). Alternatively, the family stress model describes how increasing economic resources can reduce parental stress and, in turn, increase their emotionally responsive parenting and improve the home environment(14). Broadly, it is understood that interventions to reduce household poverty will nurture children's health and development.
In the section on 'Study Eligibility' authors mention that "studies will be ineligible if there is a focus on cash transfer programme" (line # 44-45). Why so? Does cash transfer not qualify for income maximisation tool of families? Need some clarifications on this?	We are not including studies with cash transfer programmes as there is already a systematic review (see below) covering cash transfers while less is known about other integrated models in health. This is highlighted in the introduction. Siddiqi A, Rajaram A, Miller S. Do cash transfer programmes yield better health in the first year of life? A systematic review linking low-income/middle-income and high-income contexts. Archives of Disease in Childhood. 2018;103.
Why do authors consider child health and wellbeing as a secondary outcome and not primary? I guess main outcomes of the planned review should be both financial as well as health. Need some rationale of dividing the outcomes into the two categories.	Thank you for your suggestion. We have removed the division of outcomes into the two categories 'primary' and 'secondary' outcomes. This is consistent with the outcomes we have listed in the Prospero registration https://www.crd.york.ac.uk/prospero/display_record.php?RecordID=195985 Studies will have to measure at least one of the following outcomes: (1) income (change in income/earnings/ debt management); (2) other financial impact (financial literacy); (3) parental/caregiver or child health and wellbeing, including physical, mental and spiritual health, social wellbeing, developmental wellbeing of the child, parenting skills, service use, cost and harm.

VERSION 2 – REVIEW

REVIEWER	Dani Filc Ben-Gurion University of the Negev, Politic and Government
REVIEW RETURNED	19-Nov-2021

GENERAL COMMENTS	The modifications address all the points I raised.